

Hand Hygiene Policy

Purpose

The purpose of this policy is to provide instructions, indications and technique for the use of hand hygiene as the single most effective action in the prevention of transmission of infections to staff, patients and general public.

Hand hygiene includes both washing, use of adjunct alcohol based hand rubs and the care of the skin of the hands and nails. It also includes the surgical Hand Hygiene.

Scope

This policy applies to all health care providers including physicians, nurses, physiotherapists, respiratory therapists, housekeeping staff, contracted services personnel, volunteers and students.

All personnel, physicians, nurses, technicians and others who are responsible for complying with the hand hygiene policy should lead by example and call observed infractions to the attention of any offenders.

Duties and Responsibilities

-Infection Control Team(ICT) will do the following:

1. Set the example for hand hygiene practices according to the 5 Moments for Hand Hygiene when they are with patients.
2. Ensure systems and practices prescribed in this Policy are implemented and sustained
3. Ensure patients and visitors have access to the means to perform hand hygiene, with education on the 5 moments of hand hygiene and the correct technique.
4. Monitor compliance and practices described in this Policy
5. Facilitate and ensure compliance of staff with hand hygiene for every patient contact
6. Ensure that individual staff are advised at the time of any non-compliance & manage staffs who repeatedly do not comply with the 5 moments for Hand Hygiene, as outlined in this Policy and in accordance with current RH Health policies for managing allegations of misconduct.
7. Ensure that hand hygiene audits are undertaken and reported on in the ward/unit/service on a quarterly basis.
8. Ensure that hand hygiene audit results are acted on in the ward/unit/service.
9. Provide active support to all staff with regard to hand hygiene practices and compliance

10. Promote awareness to staff on hand hygiene practices.

All Healthcare workers must comply with this policy. They must educate & inform the patients about their right to ask healthcare workers to perform hand hygiene when indicated. All opportunities should be taken to educate patients' attendant & visitors on the importance of compliance with hand hygiene

-Department's directors (and head of departments (HODs) should ensure compliance of their staff to this policy and make all efforts to monitor compliance and give feedback. They should be actively planning and improving the hand hygiene compliance among their staff in a continuous manner.

Factors that may Influence the Effectiveness of Hand Hygiene Practice

1. Nails

Must be kept clean, natural and short at all times (the tips of the nails do not extend past the ends of their fingers)

Must not wear artificial nails, nail enhancements (including gel and acrylic nails), or nail polish. Artificial nails and chipped nail polish may be associated with an increase in the number of bacteria on finger nails and should not be used.

2. Hand/wrist jewelry, rings or watches

Remove when carrying out patient care

3.Sleeves

Keep out of the way during patient care activities

Long sleeves must be pulled up in order to complete proper hand hygiene

Floppy sleeves may become contaminated and must not be worn.

4. Skin/Medical Conditions/Splints

Health care providers those are unable to perform hand hygiene due to:

An injury which requires a hand splint, cast,dressing, or bandage etc. or, a skin condition (e.g. extensive cracks and bleeding from eczema, psoriasis, etc.), must immediately report these to their HODs and staff clinic and infection

prevention & Control department for consultation. If a health care provider is unable to perform effective hand hygiene, alternate, non-patient care, work arrangements need to be considered.

5. Glove Use:

The use of gloves is not a substitute for performing hand hygiene

Always perform hand hygiene after gloves removal.

Procedure

Indications for Hand Hygiene (Appendix 1)

1. Clean your hands before touching a patient.
2. Clean your hands after touching a patient and the immediate surroundings.
3. Clean your hands immediately before an aseptic technique.
4. Clean your hands immediately after an exposure risk to body fluids (and after glove removal).
5. Clean your hands after touching any object or furniture in the patient's immediate surroundings when leaving – even if the patient has not been touched.

Other Opportunities for Hand Hygiene

1. When hands are visibly soiled
2. After contact with a source of microorganisms (body fluids and substances, mucous membranes, non-intact skin, surfaces that are likely to be contaminated)
3. After removing gloves
4. When moving from a contaminated body site to a clean body site during patient care
5. Before leaving the patient's room
6. Before and after smoking, eating or preparing food
7. After bodily functions (e.g., using the toilet, blowing one's nose, sneezing)

Technique of Hand Washing (Appendix 2)

Wash hands for a minimum of 40-60 seconds

1. Remove excess jewelry
2. Select a comfortable water temperature
3. Wet hands with running water
4. Apply soap to cover all surfaces of the hands
5. Rub hands palm to palm
6. Right palm over left dorsum with interlaced fingers and vice versa
7. Palm to palm with fingers interlaced
8. Backs of fingers to opposing palms with fingers interlaced
9. Rotational rubbing of the left thumb clasped in the right palm and vice versa
10. Rotational rubbing backward and forward with clasped fingers of the right hand in the left palm and vice versa
11. Rinse the hands with running water to remove all soap residue, holding hands in upward position over sink
12. Dry the hands with a paper towel
13. Turn the faucet off with the used paper towel

Techniques of Hand rubbing (Appendix 3)

1. Use alcohol-based hand antiseptic rub for a minimum of 20-30 seconds
2. Apply to dry, visibly clean hands
3. Rub hands vigorously to apply hand antiseptic to all surfaces of hands (as in steps 5 to 10 above)
4. Allow hands to dry

NB: Use only soap and water when dealing with spore-forming bacteria (e.g., *Clostridium difficile*) and/or when your hands are visibly soiled

Care of hands

Use hand moisturizers to replace the oils lost by frequent hand hygiene procedures.

Ensure that the skin on your hands is intact. Cover non-intact skin areas with an occlusive dressing.

Do not use petroleum-based lotions, as they may interfere with glove integrity.

Medical assessment

Skin excessive dryness or cracking that result from hand hygiene soap and water or alcohol based hand rubs should be reported to staff clinic as well as infection prevention & control department.

HCWs that have exudative lesions or weeping dermatitis should refrain from all direct patient care and from handling patient care equipment until the condition resolves.

Use of gloves

Please refer to Glove Use Policy [POL/IPC/09/Vers.1.0](#)

Surgical hand hygiene (Appendix 4)

Before starting surgical hand hygiene preparation (hand scrub or hand rub) remove all jewelry and wrist watches before entering the Operating Room (OR) suite, Wash hands and arms up to the elbows with a non-medicated soap before entering the OR area.

Use a nail cleaner for the first surgical hand scrub of the day.

Surgical hand scrub with antimicrobial soap

1. Start timing and then scrub each side of each finger, between the fingers and the back and front of the hand for two minutes
2. Scrub the arms, keeping hands higher than the arms at all times
3. Wash each side of the arm from wrist to the elbow for one minute, repeating the process on the other hand and arm

4. Rinse hands and arms by passing them through the water in one direction (from fingertip to elbow), always keeping the hands above the elbows.
5. Proceed to the OR holding hands above the elbows.
6. Dry hands with a sterile towel and use aseptic technique to put on gloves.

NB:

The duration of the procedure depends on the ingredients and the manufacturer's instructions (can range from 2-6 minutes).

Surgical hands rub with alcohol-base preparation

1. Start timing
2. Use sufficient product to keep hands and forearms wet with the hand rub throughout the procedure
3. See attachment (Appendix 4) for proper technique
4. After application of the product, allow hands and forearms to dry before donning sterile gloves
5. Proceed to the Operating Theatre holding hands above the elbows.

NB:

- The duration of the procedure depends on the ingredients and the manufacturer's instructions (can range from 2-6 min) and should last until hands are dry.
- A disposable sponge or a combination of a sponge and brush has been shown to reduce bacterial counts on the hands. NB: The brush part should be used for nails only.

Patients and Visitors

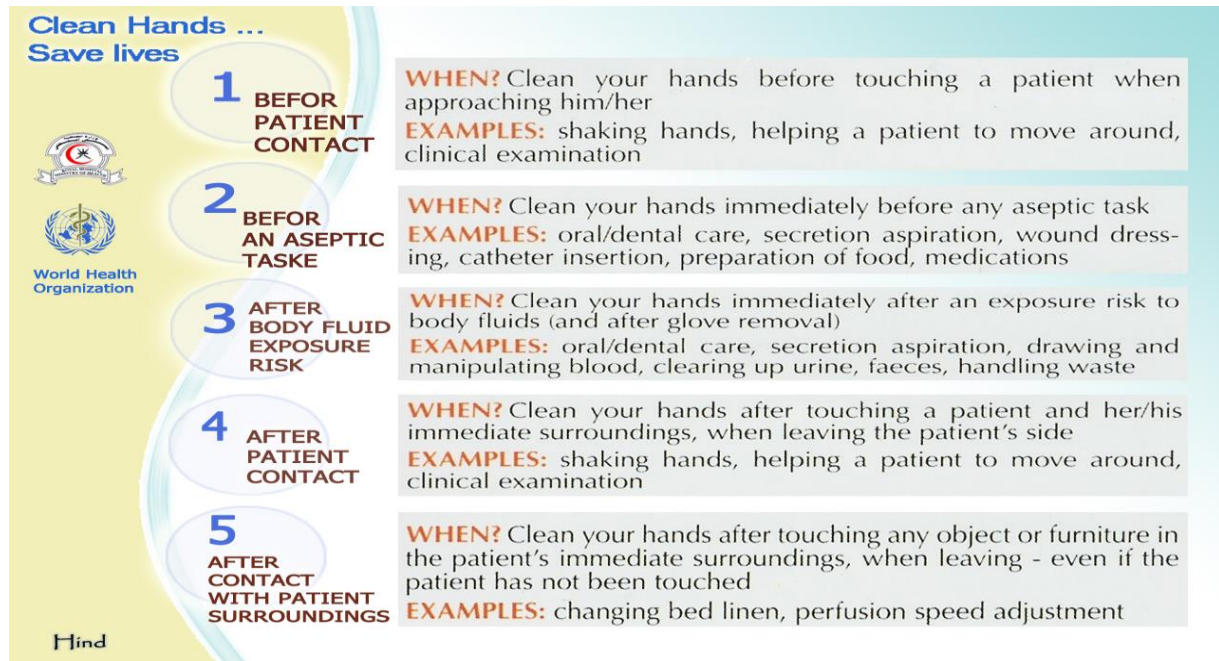
- Patients should be encouraged to comply with good hand hygiene practice. Patients if unable to use the bathroom facilities must be offered hand washing facilities at their bedside i.e. wash bowl or be given access to hand wipes. If patients are unable to perform this activity independently then staff should assist them.
- Visitors are expected to comply with good infection control practice and are encouraged to practice hand decontamination as outlined above.

- For patients nursed with Standard Isolation Procedures, visitors must decontaminate their hands before and after contact with the patient, their immediate surroundings and on leaving the room. Children may require supervision to ensure hand hygiene is adequately performed.

Monitoring Compliance and Effectiveness

- The Infection Prevention and Control team will monitor and review this policy to ensure the aims / objectives of the policy are met. The compliance and review processes will include:
- Infection Prevention & Control play an important role in hand hygiene education and audits compliance on a quarterly basis for all in patient areas (Refer to appendix 5 WHO hand hygiene observation audit tool)
- Hand hygiene audit data will be fed back to individual areas including clinical directors and Director of nursing on a quarterly basis. Data will also be report to administration.

Appendix 1: Indications for Hand Hygiene



Appendix 2: Technique for Hand Washing



Clean hands... Save lives

HOW to hand Wash??

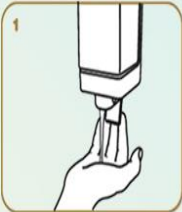
Wash hands only when visibly soiled! otherwise, use handrub!

 Duration of the entire procedure: 40-60 sec

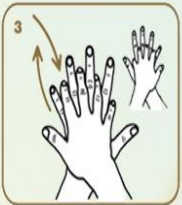


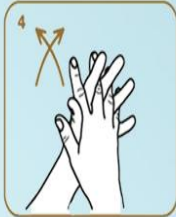
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0. Wet hand with water
- 

1. Apply enough soap to cover all hand surfaces
- 

2. Rub hands palm to palm
- 

3. Right palm over left dorsum with interlaced fingers and vice versa
- 

4. Palm to palm with fingers interlaced
- 

5. Backs of fingers to opposing palms with fingers interlocked
- 

6. rotational rubbing of left thumb clasped in right palm and vice versa
- 

7. Rotational rubbing, backwards and forward with clasped fingers of right hand in left palm and vice versa
- 

8. rinse hands with water
- 

9. Dry hands throughle with a single use towel
- 

10. Use towel to turn off faucet
- 

11. Your hands are now safe

Hind

Appendix 3: Technique for Hand Rubbing with Alcohol hand rub

How to handrub?

RUB HANDS FOR HAND HYGIENE! WASH HANDS ONLY WHEN VISIBLY SOILED!



Duration of the entire procedure: 20-30 Sec.



Apply a palmful of the product in a cupped hand, covering all surfaces.



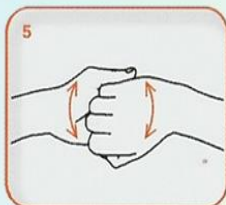
Rub hands palm to palm,



right palm over left dorsum with interlaced fingers and vice versa,



palm to palm with fingers interlaced,



backs of fingers to opposing palms with fingers interlocked,



rotational rubbing of left thumb clasped in right palm and vice versa,



rotational rubbing, backwards and forwards with clasped fingers of right hand in left palm and vice versa.



Once dry, your hands are safe.

Clean hands ... Save lives



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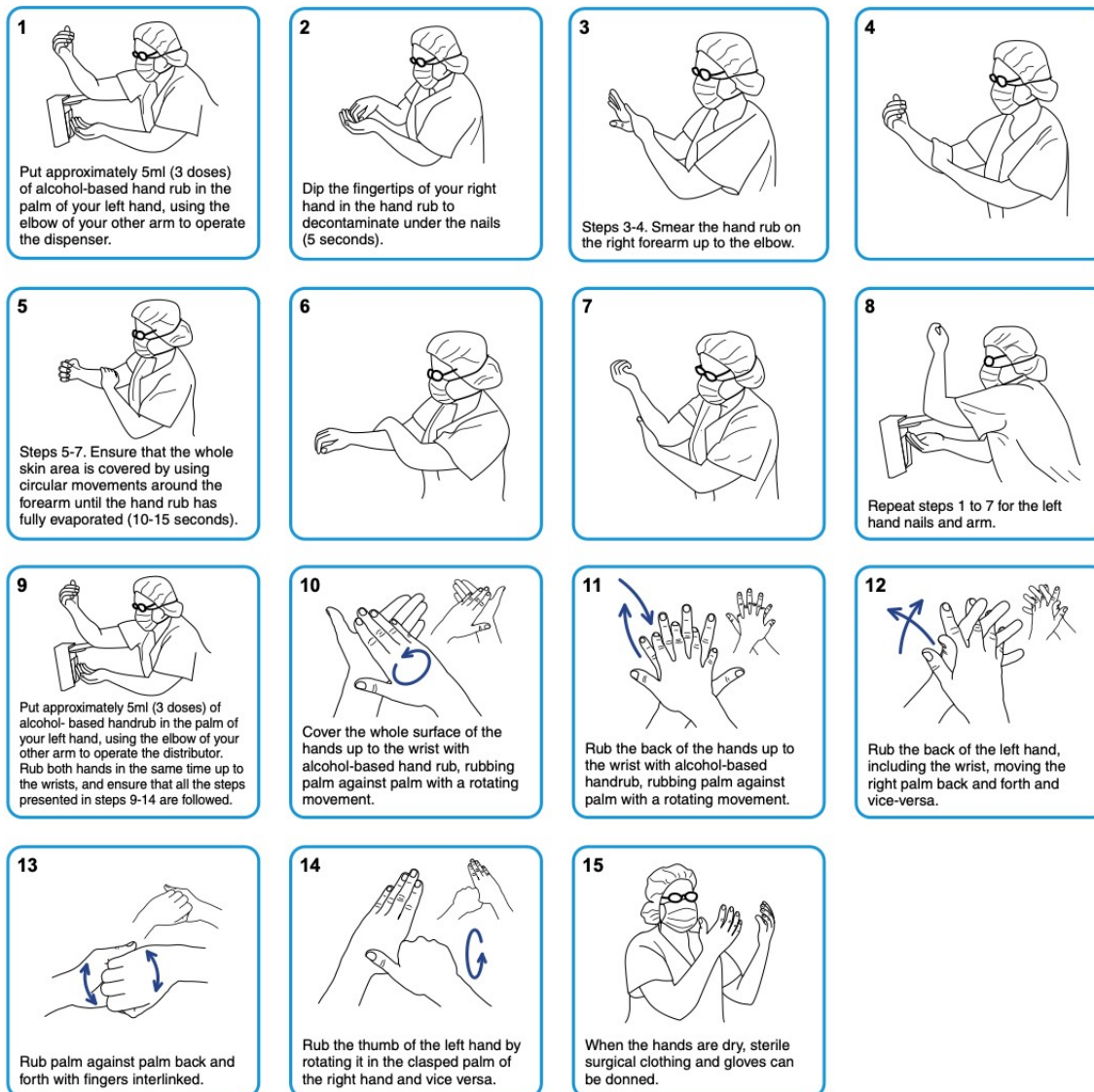
Hind

Appendix 4: Surgical Hand Hygiene

The hand rubbing technique for surgical hand preparation must be performed on perfectly clean, dry hands. On arrival in the operating theater and after having donned theater clothing (cap and mask), hands must be washed with soap and water.

After the operation when removing gloves hands must be rubbed with an alcohol-based formulation or washed with soap and water if any residual talc or biological fluids are present (e.g. if the glove is punctured).

Surgical procedures may be carried out one after the other without the need for hand washing, provided that the hand rubbing technique for surgical hand hygiene is followed (Images 1-15). Repeat the below illustrated sequence (average duration 60 seconds) according to the number of times corresponding to the total duration recommended by the manufacturer for surgical hand preparation with alcohol based hand rub



Appendix 5 WHO hand hygiene observation form



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Observation Form

Facility:		Period Number*:		Session Number*:	
Service:		Date: (dd/mm/yy)	/ /	Observer: (initials)	
Ward:		Start/End time: (hh:mm)	: / :	Page N°:	
Department:		Session duration: (mm)		City**:	
Country**:					

Prof.cat	Code	N°	Opp.	Indication	HH Action	Prof.cat	Code	N°	Opp.	Indication	HH Action	Prof.cat	Code	N°	Opp.	Indication	HH Action	Prof.cat	Code	N°	Opp.	Indication	HH Action
			1	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				1	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				1	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				1	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves
			2	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				2	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				2	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				2	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves
			3	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				3	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				3	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				3	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves
			4	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				4	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				4	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				4	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves
			5	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				5	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				5	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				5	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves
			6	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				6	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				6	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				6	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves
			7	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				7	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				7	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				7	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves
			8	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				8	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				8	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves				8	☐ bef-pat. ☐ bef-asept. ☐ aft-b.f. ☐ aft-pat. ☐ aft.p.surr.	☐ HR ☐ HW ☐ missed ☐ gloves

* To be completed by the data manager.

** Optional, to be used if appropriate, according to the local needs and regulations.

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General Recommendations

(refer to the Hand Hygiene Technical Reference Manual)

1. In the context of open and direct observations, the observer introduces him/herself to the health-care worker and to the patient when appropriate, explains his/her task and proposes immediate informal feedback.
2. The health-care worker, belonging to one of the main four following professional categories (see below), is observed during the delivery of health-care activities to patients.
3. Detected and observed data should be recorded with a pencil in order to be immediately corrected if needed.
4. The top of the form (header) is completed before starting data collection (excepted end time and session duration).
5. The session should last no more than 20 minutes ($\pm$ 10 minutes according to the observed activity); the end time and the session duration are to be completed at the end of the observation session.
6. The observer may observe up to three health-care workers simultaneously, if the density of hand hygiene opportunities permits.
7. Each column of the grid to record hand hygiene practices is intended to be dedicated to a specific professional category. Therefore numerous health-care workers may be sequentially included during one session in the column dedicated to their category. Alternatively each column may be dedicated to a single health-care worker only of whom the professional category should be indicated.
8. As soon as you detect an indication for hand hygiene, count an opportunity in the appropriate column and cross the square corresponding to the indication(s) you detected. Then complete all the indications that apply and the related hand hygiene actions observed or missed.
9. Each opportunity refers to one line in each column; each line is independent from one column to another.
10. Cross items in squares (several may apply for one opportunity) or circles (only a single item may apply at one moment).
11. When several indications fall in one opportunity, each one must be recorded by crossing the squares.
12. Performed or missed actions must always be registered within the context of an opportunity.
13. Glove use may be recorded only when the hand hygiene action is missed while the health-care worker is wearing gloves.

Short description of items

Facility:	to complete according to the local nomenclature	
Service:	to complete according to the local nomenclature	
Ward:	to complete according to the local nomenclature	
Department:	to complete according to the following standardized nomenclature:	
	medical, including dermatology, neurology, haematology, oncology, etc.	surgery, including neurosurgery, urology, EENT, ophthalmology, etc.
	mixed (medical & surgical), including gynaecology	obstetrics, including related surgery
	paediatrics, including related surgery	intensive care & resuscitation
	emergency unit	long term care & rehabilitation
	ambulatory care, including related surgery	other (to specify)
Period N°:	1) pre- / 2) post-intervention; and then according to the institutional counter.	
Date:	day (dd) / month (mm) / year (vv)	
Start/end time:	hour (hh) / minute (mm)	
Session duration:	difference between start and end time, resulting in minutes of observation.	
Session N°:	attributed at the moment of data entry for analysis.	
Observer:	observer's initials (the observer is responsible for the data collection and for checking their accuracy before submitting the form for analysis).	
Page N°:	to write only when more than one form is used for one session.	
Prof.cat:	according to the following classification:	
	1. nurse / midwife	1.1 nurse, 1.2 midwife, 1.3 student.
	2. auxiliary	
	3. medical doctor	3.1 in internal medicine, 3.2 surgeon, 3.3 anaesthetist / resuscitator / emergency physician, 3.4 paediatrician, 3.5 gynaecologist, 3.6 consultant, 3.7 medical student.
	4. other health-care worker	4.1 therapist (physiotherapist, occupational therapist, audiologist, speech therapist), 4.2 technician (radiologist, cardiology technician, operating room technician, laboratory technician, etc), 4.3 other (dietician, dentist, social worker and any other health-related professional involved in patient care), 4.4 student.
Number:	number of observed health-care workers belonging to the same professional category (same code) as they enter the field of observation and you detect opportunities.	
Opp(ortunity):	defined by one indication at least	
Indication:	reason(s) that motivate(s) hand hygiene action; all indications that apply at one moment must be recorded	
	bef.pat: before touching a patient	aft.b.f: after body fluid exposure risk
	bef.asept: before clean/aseptic procedure	aft.pat: after touching a patient
		aft.p.surr: after touching patient surroundings
HH action:	response to the hand hygiene indication(s); it can be either a positive action by performing handrub or handwash, or a negative action by missing handrub or handwash	
	HR: hand hygiene action by handrubbing with an alcohol-based formula HW: hand hygiene action by handwashing with soap and water	Missed: no hand hygiene action performed

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References

Title of book/ journal/ articles/ Website	Author	Year of publication	Page
GCC infection prevention & control Manual	GCC IC team	2013	33-38

Review / Revision History

Revision History			
Revision	Description of change		Revision year
1	Author	Dr. Zaina Al Maskari	2016
2	Reviewed by	Jayalakshmi	2018
3	Reviewed by	Dr. Amal Altai	2021
4	Reviewed by	Aisha Al Mamari	2024
5			
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